



GUIDELINE

Off-Label Psychotropic Prescribing Guide for ICAMHS Clinicians

Scope (Staff):	ICAMHS Clinicians
Scope (Area):	ICAMHS - wide

Child Safe Organisation Statement of Commitment

CAHS commits to being a child safe organisation by applying the National Principles for Child Safe Organisations. This is a commitment to a strong culture supported by robust policies and procedures to reduce the likelihood of harm to children and young people.

This document should be read in conjunction with this [disclaimer](#)

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Aim

This guideline is to be used by Infant, Child and Adolescent Mental Health Service (ICAMHS) clinicians as a quick-reference guide to determine the licensed age, and indication for psychotropic medication use in paediatric patients. This guide also contains a very brief description of the level of evidence for use of the psychotropic medication in major international clinical guidelines. It should be used in conjunction with the [Consent for Medication Use](#) form which will be included in the patient's medical record.

This guide is not intended to be used as a primary reference for medication dosages.

The guideline should also be used in conjunction with [Responsibilities of ICAMHS Clinicians](#) and [Appendix 1 – Flowchart for Assessing Appropriateness of Off-label Medicines Use](#).

Risk

All information contained in this guideline is correct at the time of publication and can be subject to change, clinicians are to ensure they have current information.

- This guideline should be read in conjunction with the relevant Product Information sheet for more detailed information as required.

Background

The *Mental Health Act (MHA) (WA) 2014* s.304 states that a record must be made of off-label prescribing in involuntary paediatric inpatients. ICAMHS clinicians are required to attain consent for the use of off-label medication from the parent when prescribing off-label treatments, irrespective of their status under the MHA 2014.

Off-label treatment means the use of registered therapeutic goods other than in accordance with the approved product information for the registered therapeutic good. If the medication is being used outside of the age group, dose, indication or route as mandated by Therapeutic Goods Administration (TGA) registration and/or official manufacturers Product Information (PI) then it constitutes off-label prescribing.

Despite off-label status of a psychotropic in Australia, there may be reasonable evidence in favour of its use. The United States Food and Drug Administration (FDA) approval status is also displayed to guide clinicians in decision making.

Abbreviations

BPAD1: Bipolar Affective Disorder 1

SCZ: Schizophrenia

BD: Behavioural Disturbance in Intellectual Disability

AUT: Autism

ADHD: Attention Deficit Hyperactivity Disorder

MD: Major Depression

OCD: Obsessive Compulsive Disorder

GAD: Generalised Anxiety Disorder

PBS: Pharmaceutical Benefits Scheme

Key point

- All information contained in this guideline is correct at the time of publication and can be subject to change.
 - This guideline should be read in conjunction with the relevant Product Information sheet for more detailed information as required. Class of Medications

Responsibilities of ICAMHS Clinicians

Clinicians must:

- Comply with recording and reporting requirements as per the s. 304 MHA 2014: Off-label treatment provided to a child who is an involuntary patient.
- Evaluate the evidence for self;
- Inform the patient and family the medication is off-label;
- Discuss any alternatives;
- Provide the patient and family with consumer product information and provide an opportunity to study same and to answer any questions; and
- Obtain written consent.

Class of Medications

Antidepressants

	Australian Licensed Use for <18 years in Psychiatry	Statewide Medicines Formulary	FDA licensed Use for <18 years in Psychiatry
Citalopram	No	2 nd line treatment MD in ≥ 8 years Management anxiety ≥ 8 years *Suicide risk warning*	No
Escitalopram	No	2 nd line treatment MD in ≥ 8 years Management anxiety ≥ 8 years *Suicide risk warning*	MD ≥12 years GAD ≥7 years
Fluoxetine	No	First line for major depression in patients ≥ 8 years. For anxiety in patients ≥ 8 years. For autism spectrum disorders; (repetitive behaviour). For bulimia nervosa	MD ≥8 years OCD ≥7 years Depressed BPAD1 in combo with olanzapine: 10-17 years
Fluvoxamine	≥8 years in OCD	For prescription under the direction of a Psychiatrist	OCD ≥8 years

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Antidepressants cont.	Australian Licensed Use for <18 years in Psychiatry	Statewide Medicines Formulary	FDA Licensed Use for <18 years in Psychiatry
Sertraline	≥6 years in OCD	First line for major depression in patients ≥ 8 years For anxiety in patients ≥ 8 years For autism spectrum disorders; (repetitive behaviour)	OCD ≥6 years
Duloxetine	No	Unrestricted	GAD ≥7 years Fibromyalgia ≥13 years
Desvenlafaxine	No	*For use as per the PBS indications and criteria* OR For prescribing under the direction of a Psychiatrist	No
Venlaflexine	No	For prescription under the direction of a Psychiatrist	No
Amitriptyline	No	Unrestricted	Depression ≥12 years
Clomipramine	No	Unrestricted	OCD ≥10 years
Nortriptyline	No	Unrestricted	Depression
Mirtazapine	No	Unrestricted	No
Vortioxetine	No	Not listed	No
Agomelatine	No	Not Listed	No

Oral Antipsychotics

	Australian Licensed Use for <18 years in psychiatry	Statewide Medicines Formulary	FDA licensed Use for <18 years in psychiatry
Aripiprazole	No	For prescription by a Paediatrician or Psychiatrist to treat children over 6 years of age with severe aggression/irritability combined with an intellectual disability and/or an autism spectrum disorder when other psycho-social interventions have failed *Assessment of risk, self-harm or suicidal tendencies to be undertaken prior to commencement, and 1-3 weeks subsequent to commencement (and if indicated, commencement at hospital). *Antipsychotic monitoring chart to be initiated and used.	SCZ ≥ 13 years Tourette's ≥ 6 years BPAD1 monotherapy ≥ 10 years BPAD1 combination with lithium or valproate ≥ 10 years AUT ≥ 6 years
Olanzapine	No	For prescription under the direction of a Psychiatrist, Neurologist or Paediatrician (prescribers should consider referring to the CAMHS Antipsychotic guidelines). For use as per the CAHS Acute Agitation and Arousal Guideline in children >6 years.	SCZ ≥ 13 years BPAD1 with depression ≥ 10 years in combination with fluoxetine BPAD1, acute mixed or manic episode ≥ 13 years
Quetiapine	BPAD1 ≥ 10 years SCZ ≥ 13 years	For prescription under the direction of a Psychiatrist, Neurologist or Paediatrician (prescribers should consider referring to the CAMHS Antipsychotic guidelines).	SCZ ≥ 13 years BPAD1 monotherapy ≥ 10 years

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Oral Antipsychotics cont	Australian Licensed Use for <18 years in psychiatry	Statewide Medicines Formulary	FDA licensed Use for <18 years in psychiatry
Risperidone	SCZ ≥ 15 years BD ≥ 5 years AUT	For prescription under the direction of a Psychiatrist, Neurologist or Paediatrician (prescribers should consider referring to the CAMHS Antipsychotic guidelines).	SCZ ≥ 13 years AUT ≥ 5 years BPAD1 ≥ 10 years
Clozapine	Treatment resistant SCZ ≥ 16 years	Not listed	No
Chlorpromazine	BD ≥ 5 years	For use under the direction of a Psychiatrist	Problem behaviour (severe) 1- ≥ 12 years
<u>Zuclopenthixol hydrochloride</u>	No	Not listed	No
Paliperidone	No	For prescription under the direction of a Psychiatrist, Neurologist or Paediatrician For use as per local hospital's Acute Agitation and Arousal Guideline in Children > 6 years. Refer to local hospital's guidelines where appropriate	SCZ ≥ 12 years
Lurasidone	SCZ ≥ 13 years	For prescribing by Paediatricians, Psychiatrists and Neurologists	BPAD1 monotherapy ≥ 10 years SCZ ≥ 13 years
Brexpiprazole	No	Not listed	SCZ ≥ 13 years
Ziprasidone	No	Not listed	No

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Oral Antipsychotics cont	Australian Licensed Use for <18 years in psychiatry	Statewide Medicines Formulary	FDA licensed Use for <18 years in psychiatry
Haloperidol	Tourettes ≥ 3 years	Unrestricted	Tourettes ≥ 3 years SCZ ≥ 3 years Hyperactive behaviour ≥ 3 years Problematic behaviour (severe) ≥ 3 years

Parenteral Antipsychotics

	Australian Licensed Use for <18 years in psychiatry	Statewide Medicines Formulary	FDA licensed Use for <18 years in psychiatry
Olanzapine	No	For use under the direction of a Psychiatrist, Neurologist or Paediatrician For use as per the CAHS Acute Agitation and Arousal Guideline in children >6 years	No
Zuclopenthixol acetate	No	For use under the direction of a Psychiatrist	No
Droperidol	No	Unrestricted	No
Haloperidol lactate	Tourettes ≥ 3 years	Unrestricted	Tourettes ≥ 3 years SCZ ≥ 3 years Hyperactive behaviour ≥ 3 years Problematic behaviour (severe) ≥ 3 years

Depot Antipsychotics

	Australian Licensed Use for <18 years in psychiatry	Statewide Medicines Formulary	FDA licensed Use for <18 years in psychiatry
Aripiprazole	No	For prescription by a Paediatrician or Psychiatrist to treat children over 6 years of age with severe aggression /irritability combined with an intellectual disability and/or an autism spectrum disorder when other psycho-social interventions have failed. *Assessment of risk self-harm or suicidal tendencies to be undertaken prior to commencement, and 1-3 weeks subsequent to commencement (and, if indicated, commencement at hospital). *Antipsychotic monitoring chart to be initiated and used.	No
Olanzapine pamoate	No	Not listed	No
Risperidone	No	Not listed	No
Paliperidone palmitate	No	*For use as per PBS indications and criteria* under the direction of a Psychiatrist Refer to local hospital's guidelines where appropriate.	No

Mood Stabilisers

	Australian Licensed Use for <18 years in psychiatry	Statewide Medicines Formulary	FDA licensed Use for <18 years in psychiatry
Lamotrigine	No	*For use as per PBS indications and criteria* For borderline personality disorder under the direction of a Psychiatrist. For bipolar depression under the direction of a Psychiatrist	No
Lithium	≥12 years	To be prescribed under the direction of a Child and Adolescent Psychiatrist. Refer to local hospital guidelines where appropriate.	Immediate release: BPAD1 maintenance & acute ≥7 years Controlled release: BPAD1 maintenance & acute ≥12 years
Carbamazepine	No	Unrestricted	No
Sodium Valproate	No	Unrestricted	No

Stimulants

	Australian Licensed Use for <18 years in psychiatry	Statewide Medicines Formulary	FDA licensed Use for <18 years in psychiatry
Methylphenidate	ADHD $\geq$ 6 years	*For use as per PBS indications and criteria for Attention Deficit Hyperactivity Disorder (ADHD) as per local policy* Prescription may only be by an Approved Specialist as per WA Monitored Medicines Prescribing Code.	ADHD $\geq$6 years Narcolepsy $\geq$6 years
Dexamfetamine	ADHD $\geq$ 3 years	*For use as per PBS indications and criteria for Attention Deficit Hyperactivity Disorder (ADHD) as per local policy* Prescription may only be by an Approved Specialist as per WA Monitored Medicines Prescribing Code.	ADHD $\geq$3 years Narcolepsy $\geq$6 years
Lisdexamfetamine	ADHD $\geq$ 6 years	*For use as per PBS indications and criteria for Attention Deficit Hyperactivity Disorder (ADHD) as per local policy* Prescription may only be by an Approved Specialist as per WA Monitored Medicines Prescribing Code.	ADHD $\geq$ 6 years

Benzodiazepines

	Australian Licensed Use for <18 years in psychiatry	Statewide Medicines Formulary	FDA licensed Use for <18 years in psychiatry
Midazolam (oral pre-filled syringe & ampoules)	No	Unrestricted	No
Clonazepam (tablets, drops & ampoules)	No	Unrestricted	No
Lorazepam (tablets)	Anxiety ≥ 16 years	Unrestricted	Anxiety ≥ 12 years Insomnia due to anxiety or situational stress ≥ 12 years
Lorazepam IM (SAS)	No	*Treatment must meet requirements for Special Access Scheme (SAS) supply* Can only be used where local hospital guidelines exist for the management of acute agitation and arousal for older adolescent mental health inpatients aged 16-17 years.	No
Temazepam	Insomnia (short term) ≥ 16 years	Unrestricted	No
Diazepam oral	Anxiety (short term) ≥ 6 months Acute alcohol withdrawal	Unrestricted	Anxiety ≥ 6 months

Other

	Australian Licensed Use for <18 years in psychiatry	Statewide Medicines Formulary	FDA licensed Use for <18 years in psychiatry
Nicotine Replacement Patch	≥12 years	Unrestricted	No
Clonidine oral	No	Unrestricted	ADHD ≥6 years
Atomoxetine	ADHD ≥6 years	*For use as per the PBS indications and criteria*	ADHD ≥6 years
Guanfacine	ADHD >6 years	*For use as per the PBS indications and criteria as per local hospital's policy*	ADHD >6 years
Melatonin (tablets, liquid)	No	Dose is not to exceed 6mg nocte. Restricted prescribing under the direction of a Neurologist, Neurorehabilitation Specialist or Sleep Physician for refractory insomnia associated with neurodevelopmental disorders, acquired brain injury or cortical blindness. Supply will be initially restricted to 3 months treatment. An outcome report to the DTC will be required for ongoing supply. For prescribing to patients in intensive care. For prescribing to Perth Children's Hospital (PCH) ward psychiatric inpatients. Psychiatric patients may have one month supply on discharge. No ongoing supplies are authorised. Existing patients treated prior to June 2017 can continue therapy however prescribers should review these patients for ongoing efficacy.	No

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Other cont	Australian Licensed Use for <18 years in psychiatry	Statewide Medicines Formulary	FDA licensed Use for <18 years in psychiatry
Zolpidem	No	For prescription by a Paediatrician for the treatment of adolescents with rebound insomnia.	No
Zolpiclone	No	Not listed	No

References and related external legislation, policies, and guidelines
1. WALW Mental Health Act 2014
2. MIMS Australia 2022, Various monographs viewed 3 February 2022, https://www.mimsonline.com.au .
3. Micromedex 2020, Various monographs viewed 3 February 2022, https://www.micromedexsolutions.com
4. National Safety and Quality Health Service (NSQHS) Standards Medication Safety Standard National Standards User Guide for Health Services Providing Care for People with Mental Health Issues
5. NSQHS Standards Medication Safety Standard (4) .
6. Appendix 1. Reproduced and adapted from: Gazarian M et al. Off-label use of medicines: consensus recommendations for evaluating appropriateness. MJA 2006; 185: 544-548. © Copyright 2006.

Related CAHS internal policies, procedures and guidelines <i>(if required)</i>
ICAMHS Medication Management – Specialised and Community Child and Adolescent Mental Health Service Policy
CAHS Clinical Guidelines
CAHS Medication Safety Policy
CAHS Use Of Medications For Other Than Manufacturers Approved Indications Policy
CAHS Consent to Treatment Policy
CAMHS Consent Guideline

Useful resources (including related forms) <i>(if required)</i>
ICAMHS Medication Safety Toolbox
Psychotropic Medication Handbook
MIMS Online: Approved Product Information Database
PCH Pharmacy Intranet Page

CAMHS Psychotropic Medication – Monitoring Adverse Physical Health Effects Policy
CAMHS Antipsychotic Physical Health Monitoring Form (MR505.01)
CAMHS Consent for Medication Use Form

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Appendix 1: Flowchart for Assessing Appropriateness of Off-Label Medicines Use

REMINDER: In ICAMHS we are required to use the ICAMHS Consent for Medication Use form.

